

Patient ID: SallenPeriod: February 6 to March 7, 2024Report Date: March 8, 2024Coverage: 502/735h (68%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours

Last 24 Hours: within normal range			
Vital Stat	Avg	Min	Max
Heart Rate	58 bpm	46 bpm	83 bpm
Breathing	18 brpm	7 brpm	44 brpm

Limited data in the final 24h (17/24h); values reflect available readings.

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

Patient clinical status over 31 days from Feb 06 to Mar 07

						HR	Breathing
Elevated Breathing	Elevated Breathing	Elevated Breathing	High Breathing	High Breathing	Very High Breathing		
Feb 18 to Feb 24 (7d)	Feb 28 to Feb 29 (2d)	Mar 03 to Mar 04 (2d)	Feb 22 to Feb 23 (2d)	Mar 03 to Mar 03 (1d)	Mar 03 to Mar 03 (1d)		

22 episodic events Detected, spanning 23h total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
22	23h	Elevated Breathing	<1	68%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 03	4 PM to 5 PM	1h	1	Very High Breathing	41	28	44	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	3 PM to 4 PM	2h	2	High Breathing	31	22	43	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 03	10 PM to 11 PM	1h	1	High Breathing	31	13	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 18	7 PM to 8 PM	8h	8	Elevated Breathing	27	9	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Feb 28	10 PM to 11 PM	6h	5	Elevated Breathing	27	9	54	Check SpO2 and lung sounds; assess for fluid overload or early infection
Mar 03	3 PM to 4 PM	2h	2	Elevated Breathing	26	16	43	Check SpO2 and lung sounds; assess for fluid overload or early infection

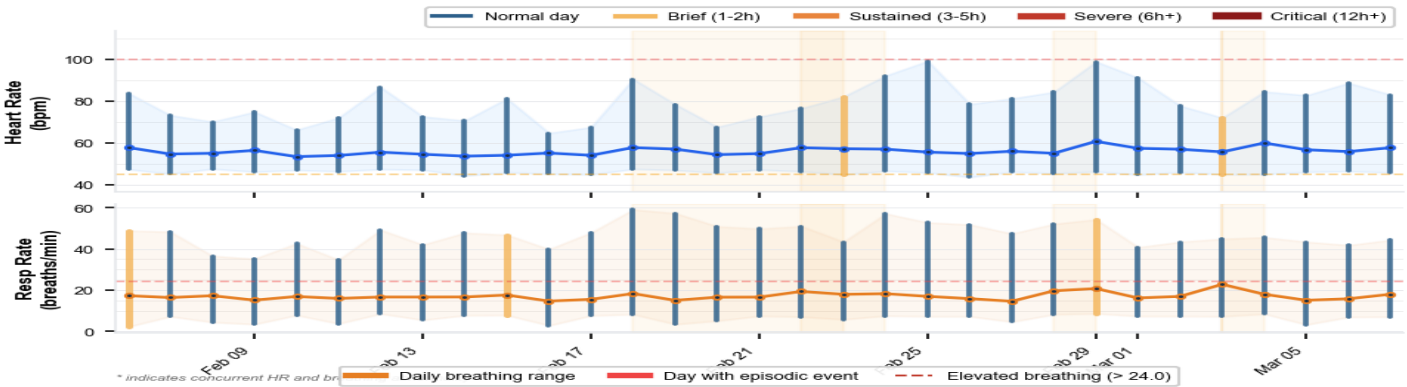
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 57 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 43 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 41 brpm, peak 44 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 18 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

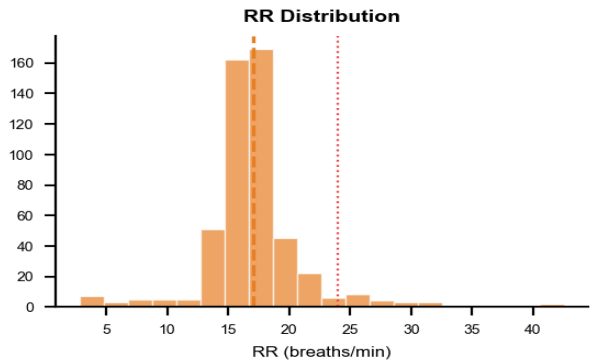
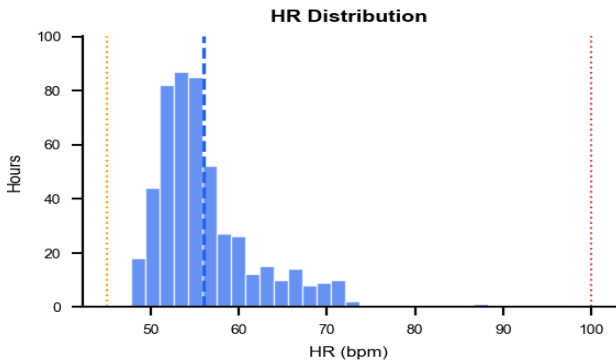
Clinical Pattern Observations:

- 1. Breathing variability: RR spread 10 brpm (12 to 23).
- 2. Clustered pattern: Episodic events on 5 of 31 days (16%).

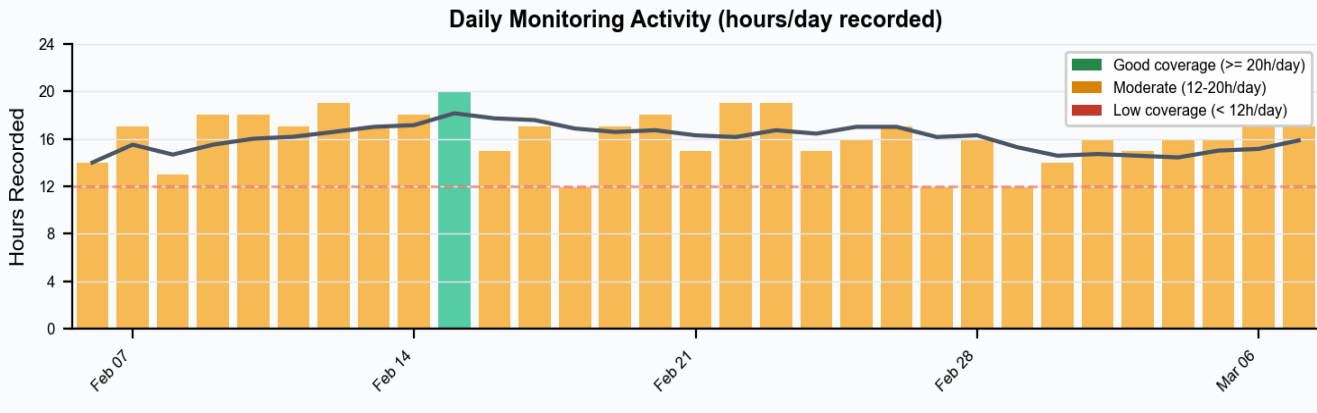


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.